

Fraud and Abuse Detection Policy

Policy ID: POL-FRAUD-002 | Version: 1.0 | Effective: 2024-01-01

1. Purpose

This policy establishes thresholds and procedures for identifying potentially fraudulent or abusive billing patterns by providers and patients. Unusual claim frequency, provider risk scores, and outlier billing patterns are primary indicators.

2. Fraud Indicators

Rule	Threshold / Condition	Action
High Claim Frequency	claim_frequency > 10 per period	Flag for investigation
Provider Risk Score	provider_risk_score > 0.7	Secondary review
High Provider Volume	provider_claim_count > 400	Pattern analysis required
Repeated High Billing	Multiple claims with high_cost_flag = 1	Audit triggered
Outlier Combination	High frequency + high cost + high risk	Immediate escalation

3. Provider Risk Score

Each provider is assigned a risk score (provider_risk_score) between 0 and 1 based on historical claim patterns, denial rates, and billing behaviour. Scores above 0.7 indicate a high-risk provider. Claims from high-risk providers are subject to enhanced scrutiny regardless of the individual claim amount.

4. Claim Frequency Abuse

Abnormally high claim submission frequency for a single patient or provider within a billing period is a strong indicator of abuse. claim_frequency represents the number of claims submitted. Values above defined thresholds trigger automatic review.

5. Provider Claim Volume

provider_claim_count reflects the total number of claims submitted by a provider. Very high volumes combined with elevated risk scores suggest systematic billing issues and require pattern-level analysis rather than per-claim review.

6. Escalation Procedure

- Flag all claims from providers with risk_score > 0.7 for enhanced review.
- Investigate providers with claim_count > 400 and multiple high_cost_flag claims.
- Escalate to fraud investigation unit when claim_frequency > 10 and risk_score > 0.6.
- Place provider on watch list upon confirmed abuse pattern.
- All escalations must be documented with claim IDs and triggering indicators.